AAHOMECARE
American Association for Homecare

Submitted Electronically: ***Regulations.gov***

June 16, 2025

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013.

Re: CMS Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Introduction

The American Association for Homecare (AAHomecare) is the national association representing durable medical equipment, prosthetics, orthotics and supplies (DMEPOS) suppliers, manufacturers, and other stakeholders in the homecare community. Our members are proud to be part of the continuum of care that assures beneficiaries, and other patients receive cost effective, safe, and reliable home care products and services. AAHomecare is pleased to submit these recommendations in response to the above-captioned request for information (RFI).

The following comments in response to the RFI outline key recommendations to enhance health technology use across the DMEPOS industry, with a focus on both traditional Medicare and Medicare Advantage (MA) plans, as well as Medicaid managed care organizations (MCOs). These recommendations include requiring ePrescribing for DMEPOS and implementing greater transparency measures, especially in regards to utilization data and the use of artificial intelligence. Improving efficiency and transparency in these areas will support timely service delivery, strengthen program integrity, and reduce unnecessary administrative burdens across all payer types.

AAHomecare Requests That CMS Take Steps to Mandate ePrescribing for All DMEPOS Orders

DMEPOS ePrescribing is an electronic ordering method that allows the prescriber to send a DMEPOS order to a supplier. ePrescribing has many benefits including (i) streamlining the prescription process; (ii) improving CMS' ability to audit documentation for DMEPOS claims; and (iii) reducing costs for prescribers, suppliers and the federal government. AAHomecare believes that many DMEPOS suppliers would invest in ePrescribing platforms if there was a clear CMS policy stating that ePrescribing is an acceptable means of creating, transmitting and collecting required documentation. CMS guidance must state that all electronic documentation must meet the same requirements as other documentation requirements. If CMS does not believe it has the authority to do so, it should seek the authority from Congress.

Current State of DMEPOS ePrescribing

There are currently approximately four predominant ePrescribing platforms utilized by the DMEPOS industry, all offering robust services and integration capabilities. Feedback from the supplier community

on incorporating ePrescribing into their operations has been tremendously positive, with all reporting significant improvements in the efficiency of the ordering process. However, the utilization of ePrescribing platforms within the DMEPOS industry represents just a fraction of total orders, leaving many still reliant on a fully paper-based process. Although CMS has the authority to encourage ePrescribing for DMEPOS orders, the Medicare Program Integrity Manual needs to be updated to reflect the use of a revised definition of allowed templates.

ePrescribing Enhances Coordination of Care

ePrescribing facilitates seamless, real-time communication between healthcare providers and DMEPOS suppliers, enhancing coordination and continuity of care. By integrating with electronic health records (EHRs) and other health IT systems, enabling interoperability and comprehensive patient management.

Embracing ePrescribing on a wider scale holds the potential to elevate patient access to care in a timely manner. Wider adoption can improve reducing delays, minimizing documentation errors, and ensuring timely delivery of medically necessary equipment and supplies. By embedding documentation requirements into the ordering workflow, ePrescribing supports providers and DMEPOS suppliers to adhere to Medicare requirements, bolsters accountability and transparency, thereby serving as a safeguard against fraud and abuse.

ePrescribing Supports Program Integrity

ePrescribing can be a powerful tool in combating fraud and abuse by establishing transparent processes for generating and recording prescription orders. Through authentication measures and validation checks, these systems verify the integrity of transactions, minimizing the risk of unauthorized activities. The platforms available today also have analytics that can flag suspicious patterns such as unusually high ordering volumes, repeated orders for the same items, or orders outside a physician's typical specialty. This enables real-time intervention and compliance coaching.

The availability of such technology that can allow all stakeholders to effortlessly access and track logged orders from initiation to fulfillment encourages accountability and good behavior throughout the ordering process. From physicians to supplier intake staff, the ability to track the order start to finish, and understand that it may be reviewed or audited, encourages improved compliance behavior across the ecosystem.

In addition, ePrescribing promotes clarity and completeness of orders, which helps with reducing both unintended errors (“fraud-by-error”) as well as intentional abuse. With paper-based orders, intake staff regularly contend with illegible handwriting, incomplete forms, and vague clinical documentation. ePrescribing on the other hand, standardizes the order submission, encourages orders to be complete and required information to be captured in a clear format.

ePrescribing Improves Auditability

Comprehensive audit trails generated by ePrescribing platforms enable auditors to effectively track and investigate any potential instances of fraud or abuse. Every action in an ePrescribing system generates a digital footprint—including who ordered what, when, and for whom. This enables both proactive fraud detection and retroactive auditing, supporting both compliance teams and external auditors (e.g., OIG). ePrescribing creates a digital trail of actions, making it easier to track and audit orders for compliance, billing, and regulatory purposes.

ePrescribing platforms also provide support for DME suppliers' compliance officers with tools that help them identify and deter bad actors within their organizations. The structured, timestamped, and

authenticated nature of ePrescribing records makes it easier for CMS, auditors, and the OIG to verify order legitimacy—strengthening trust in DME processes.

ePrescribing Promotes Efficiency
ePrescribing streamlines administrative processes, reduces paperwork and eliminates the need for manual/paper handling, which saves time and resources for all stakeholders. ePrescribing eliminates this loophole by requiring real-time, user-authenticated submission.

Additional Recommendations on ePrescribing
For a successful implementation that does not add additional burden for suppliers, CMS would need to issue guidance clarifying that ePrescribing applications are considered an extension of the prescriber's medical records. When the prescriber, or their designee, is using ePrescribing to order DMEPOS, that electronic record must be considered to be part of the medical record because it is being created and communicated by or on behalf of the prescriber.

In addition, AAHomecare requests CMS to make the oxygen clinical data elements (CDEs) that were developed by the Mettles pilot to be publicly available for use. CDEs will help guide physicians in answering structured questions and will result in contractor nurse reviewers not having to rely on notes from the medical record. Adopting the CDEs, which were approved by CMS and DME MACs, will improve the consistency and objectivity of the reviews.

Medicare Advantage Plans Should be Required to Publish Data Demonstrating They Provide Sufficient Access to Care

CMS should require Medicare Advantage (MA) plans to establish clear network adequacy criteria by DMEPOS product category and by geographic area to ensure there is real patient choice. For example, some DMEPOS suppliers only provide respiratory items and services while others only provide complex rehab technology (CRT) items and services. There should be multiple DMEPOS suppliers providing the same product category in a geographic area. CMS and/or the MA plans should establish metrics to determine when network adequacy has been met for each product category in the DMEPOS space. CMS currently has established time and distance requirements for many other provider types (e.g., hospitals, skilled nursing facilities, physicians and home health agencies). CMS should track and analyze these metrics on an annual basis to determine whether there are access to care issues. AAHomecare recommends that MA plans be required to include these network adequacy metrics in their published in their provider directories. AAHomecare welcomes the opportunity to work with CMS to develop metrics that would ensure access to care.

When a DMEPOS supplier identifies a potential access issue, there should be a clear channel within CMS for DMEPOS suppliers to escalate concerns. In the DMEPOS market, access issues can be due to a lack of competition in a market for a particular product category, resulting in essentially no patient choice of supplier. In lieu of a MA plan's attestation of access to care, CMS should require MA plans to confirm compliance via submission of data that meets prescribed access to care metrics, such as:

- Beneficiary satisfaction, complaints, and
- The number of in-network DME suppliers, by product category and by geography. The number should be based upon the number of in-network DME suppliers that have submitted claims within the last 12 months to ensure that only active suppliers are counted.

If an MA plan falls short in meeting any of these metrics, it must add providers to its network to fill the deficiencies within a specified timeframe.

MA Plans Should be Required to Publicly Disclose and Report Whether They Are Using Any Form of Artificial Intelligence (AI) as Part of the Claims Processing or Prior Authorization processes.

The algorithms and AI technology standards should be available for free to the public. MA plans often utilize AI for claims processing and PA processes, but these systems often lack the clinical nuance necessary to accurately evaluate complex medical records. This can result in inappropriate denials or delays in medically necessary services and poses significant risks to patient care and provider burden. To ensure accountability and protect beneficiaries, especially for patients with chronic or complex conditions, CMS should require MA Plans to disclose and report their use of AI.

MA Plans Should Be Required to Publicly Disclose and Report All Prior Authorization Statistics for DMEPOS Items, by Product Category.

In addition, if an MA plan utilizes third party administrators (TPAs), or if an MA plan owns a DME supplier, those TPAs should be required to report the same metrics, including:

- The number of PAs requested,
- The number of PAs approved,
- The number of PAs denied,
- Number of PAs that are partially approved or denied,
- The number of PAs appealed,
- The number of PAs that are overturned after appeal, and at what level those reversals occurred,
- The total number of PAs approved and denied within prescribed timeline, such as quarterly, and
- The number of PAs that are denied based upon technical versus medical need reasons.

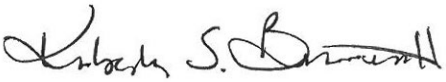
FOIA Data Should Be Available for Medicare Advantage Plans and Medicaid Managed Care Plans.

While state Medicaid programs and Medicare Fee-For-Service are subject to the Freedom of Information Act (FOIA), the same level of transparency is not required of MA plans and Medicaid managed care organizations (MCOs), despite the fact that a majority of beneficiaries are either enrolled in a MA or MCO. This discrepancy allows critical data to be withheld from public access, limiting stakeholders' ability to assess beneficiary access, monitor program integrity, and identify opportunities to improve care. CMS should require MA plans and MCOs to make data publicly available in a manner consistent with how Medicare claims data is accessible through FOIA.

Conclusion

Thank you for the opportunity to provide this information. If you have any questions, or would like further information, please contact me at kimb@aahomecare.org

Sincerely,



Kim Brummett
Senior Vice President, Regulatory Affairs
American Association for Homecare